



PROCEDURE

Goals of Patient Care

Scope (Staff):	NMHS Mental Health Inpatient Clinical Staff
Scope (Area):	NMHS Mental Health Inpatient Services

1. Aim

The aim of this document is to provide clinicians with direction on the implementation of the Goals of Patient Care (GoPC) process in North Metropolitan Health Service (NMHS) mental health settings. GoPC establishes and documents the most appropriate, agreed goals of patient care that apply in the event of a patient's clinical presentation and/or deterioration.

This procedure provides clinicians with additional information to support the implementation of the [NMHS GoPC Policy](#) and to assist clinicians in understanding policy requirements as applied to inpatients (18 years and over) in mental health settings.

2. Background

The Procedure has been developed to provide clinicians with operational information pertaining to the [NMHS GoPC Policy](#). The Department of Health WA directed inpatient services to implement the GoPC in all inpatients services using the WA GoPC Form (MR00H.1). The WA Department of Health state-wide GoPC Form (MR00H.1) Form replaces the Not For Resuscitation Form previously used in inpatient services.

3. Risk

Non-compliance with this procedure may result in a breach of legislative or mandatory requirements and will impact on the following risk categories:

Culture	☐	Organisational Development	☐
Emergency Management	☒	Policy & Legislative Compliance	☒
Fiscal Responsibility	☐	Quality of Patient Care	☒
Governance & Accountability	☐	Research & Innovation	☐
Information Communications Technology	☐	Staff Safety & Wellness	☐
Facility Management	☐	Workforce Planning	☐
Medical Equipment	☐	Other	☐

4. Definitions

Advance Care Directives	Advance Care Directives in Western Australia include Advance Health Directives and the Enduring Power of Guardianship documents. Any person with decision-making capacity is able to make their own decisions, including decisions related to life prolonging options. All
--------------------------------	--

	adults are presumed to have capacity unless there is evidence to suggest otherwise.
Advance Care Planning (ACP)	The process of planning for future health and personal care whereby the person's values, beliefs and preferences are made known so they can guide decision-making at a time when that person cannot make or communicate their decisions.
Advance Health Directive (AHD)	Advance Health Directive is an instrument recognised under the Guardianship and Administration Act 1990 which records a competent adult's decisions about possible future treatment. Recorded decisions are then followed when the make of the AHD can no longer make or communicate their decisions.
Carer	Carers provide unpaid care and support to family members and friends who have a disability, mental illness, chronic condition, terminal illness, an alcohol, or other drug issue or who are frail aged (Carers Australia, 2015).
End-of-Life (EOL) Care	End of life care helps all those with advanced, progressive, incurable illness to live as well as possible until they die. It enables the supportive and palliative care needs of both patient and family to be identified and met throughout the last phase of life and into bereavement. It includes management of pain and other symptoms and provision of psychological, social, spiritual and practical support.
Enduring Power of Guardianship (EPG)	An instrument recognised under the Guardianship and Administration Act 1990 in which a person nominates a competent adult to be an Enduring Guardian to act on their behalf when they are no longer able to do so.
Goals of Patient Care (GoPC)	A process which prompts and facilitates proactive shared decision making between clinicians and patients and/or person responsible/family/carer(s) to ensure that the treatment provided is aligned to the patient's preferences, needs, values, and wishes. It establishes and documents the agreed goals of patient care that will apply in the event of the patient's clinical presentation and/or clinical deterioration. Medical goals of care may change over time particularly as the patient enters the terminal phase and during end-of-life care.
Goals of Patient Care (GoPC) Form	In this Procedure, this refers to either the paper-based or the electronic GoPC Form unless clearly specified.
Goals of Patient Care (GoPC) electronic form (eForm)	The Goals of Patient Care (GoPC) electronic form (eForm) is the preferred clinical documentation. However, when this is not available, the paper-based Goals of Patient Care Summary Form (MR00H.1) is to be used. The GoPC eForm is stored electronically then published

	which must be then printed and placed in the patient's healthcare records.
Goals of Patient Care Summary Form (MR00H.1)	The Goals of Patient Care (GoPC) electronic form (eForm) is the preferred clinical documentation. However, when this is not available, the paper-based Goals of Patient Care Summary Form (MR00H.1) is to be used. The GoPC eForm is stored electronically then published which must be then printed and placed in the patient's healthcare records.
Senior Medical Officer	Refer to registrars or above in this Procedure.

5. Key Points

The GoPC establishes the most medically appropriate and agreed goals of care that will apply in the event of physical health deterioration, during an episode of care in an inpatient mental health service.

GoPC is a clinical process that facilitates proactive and timely shared discussion and decision-making between the medical practitioner, the patient, family, carer or guardian, regarding the wishes of the patient in the event that their medical condition deteriorates.

6. Equipment

- The Goals of Patient Care Form (MR00H.1)

7. Process

The GoPC Procedure describes the process for engaging with the patient, the family, carer or Guardian:

1. The GoPC Procedure describes a framework for having a conversation that includes plans for emergency care and treatment if the person's health deteriorates rapidly or unexpectedly that may be difficult to initiate without a clear framework to guide actions.
2. The GoPC Procedure provides information for clinicians about the documentation requirements including the completion of the GoPC Form as applied to adult inpatients (18 years and over).
3. The GoPC Form may be used in other health facilities if the patient is transferred during that episode of care.
4. If at the time of transfer the patient has a current GoPC Form in their file, it is photocopied and clearly identified as part of the transfer documentation that accompanies the patient.

-
5. It is important to schedule time with the patient, family, carer or nominated persons when developing GoPC to allow time for clarification and check for understanding.
 6. Once the agreed goals of care have been documented on the GoPC Form (MR00H.1) the Medical Officer **must read** the plan back to the patient, next of kin, family or nominated person to ensure that they understand the contents of the GoPC and what this means for the patient.

8. Timeline for GoPC process

- The GoPC Form should be completed as early as possible in the patient journey. This includes, where possible, outpatients or pre-assessment clinics. For emergency admissions the organisation-wide goal is to complete the GoPC Form within 48 hours of admission for identified patients. However, if clinical deterioration is likely or urgent interventions are planned then the GoPC Form should be completed at the earliest opportunity.
- Senior Medical Officers who have a significant role in any stage of the patient journey should consider completing a GoPC Form. In the absence of a completed form the default will be full resuscitation.

9. GoPC for Culturally and Linguistically Diverse patients

Multidisciplinary teams collaborate with individuals and families from culturally and linguistically diverse (CaLD) backgrounds to improve access to culturally appropriate healthcare. This includes providing a culturally safe environment where there is collaboration and respectful dialogue when communicating with patients, families and nominated persons. Each person needs to be assessed as an individual as there are differences within cultures and individual preferences that need to be understood.

10. Interpreter Services

Clinicians access [Interpreter Services](#) to facilitate communication if this is required when developing GoPC. Official interpreters must be used in all situations concerning the patient's medical condition and in discussions relating to GoPC. Clinicians will accommodate the needs of patients from CaLD backgrounds by acknowledging difference in attitudes to illness and treatment for different cultures and by respecting cultural diversity and the significance of spiritual beliefs in relation to conversations about treatment options and GoPC.

11. GoPC for Aboriginal patients

Multidisciplinary Teams in Mental Health inpatients services are committed to collaborating with Aboriginal people, their families, and communities to improve access to culturally appropriate healthcare by providing a culturally safe environment where there is collaboration and respectful dialogue around care planning and delivery.

Mental Health Clinicians will accommodate the needs of Aboriginal patients. This requires understanding of the importance of providing support for Aboriginal individuals, their families

and the Aboriginal community when completing GoPC Care with an expectation that clinicians will undertake the following:

- Acknowledge the Aboriginal people of the many traditional lands and language groups of Western Australia and commit to providing culturally secure services to support the health and wellbeing of Aboriginal people.
- Demonstrate their obligation to ensure that the needs of Aboriginal patients and their families are considered and incorporated into practices that are culturally responsive. At critical times, such as starting conversations about having Goals of Care in place for Aboriginal patients it is important that clinicians provide information that is culturally respectful, non-discriminatory and recognises the importance of demonstrating cultural sensitivity when interacting with individuals, their families, and communities.
- Recognise that Aboriginal communities have different beliefs, rituals, and practices to assist them in responding to end-of-life experiences and these vary across Aboriginal communities. To meet the cultural needs of Aboriginal groups it is important that clinicians obtain information about the end-of-life practices specific to the individual, their family and community. This information may be obtained through identifying which language group the patient belongs as knowing the language group will provide a starting point for the next steps in developing the GoPC.

If staff seek specific cultural information from an external service, they should only disclose gender and cultural identity but not the name or details of the patient. Clinicians access [Aboriginal Interpreter Services](#) to facilitate communication if this is requested and official interpreters must be used in all situations concerning the patient's medical condition and in discussions relating to GoPC to maintain confidentiality.

Once the agreed GoPC have been documented on the GoPC Form (MR00H.1) the Medical Officer must read the plan back to the patient, next of kin, or family to ensure that they understand the contents of the GoPC and what this means for the patient.

12. GoPC Capacity Assessment

If the patient does not have capacity to determine their goals of care, refer to the [WA Health Consent to Treatment Policy](#) and **(Appendix A)** 'Hierarchy of decision-makers for treatment' for further information.

Goals of care are completed by medical practitioners but should align with the preferred health outcomes and treatment decisions made by the individual (to the capacity they have to participate in shared decision-making).

The person may or may not have previously completed an advance care directive. Where an advance care directive has been completed, and the individual no longer has decision-making



capacity, the goals of care should reflect the advance care directive and should include a discussion with the person's substitute decision-maker.

13. GoPC Form (MROOH.1)

The GoPC process should be implemented and a GoPC Form completed as a matter of priority for patients in the groups identified as follows:

- Patients with an Advance Health Directive (AHD) or an Advance Care Plan (ACP) with health-related instructions
- Patients with advanced, life limiting conditions
- Patients admitted with suspected or confirmed COVID-19 illness
- Patients that meet clinical indicators (**Appendix B**) for poor or deteriorating physical health as per the Supportive and Palliative Care Indicators Tool (SPICCTM) criteria
- Patients that meet the clinical indicators of one or more life limiting conditions as per the SPICCTM criteria (**Appendix B**).

For patients with identified Advanced Health Directives (AHDs) or Enduring Power of Guardianship (EPG) refer to the NMHS Advance Health Directive Policy and Enduring Power of Guardianship Guideline.

If the AHD is no longer representing the agreed GoPC during their hospital admission, the AHD may be revoked (refer to the NMHS Advance Health Directive and Enduring Power of Guardianship Guideline). The GoPC Form is completed to reflect the amended treatment plan in place for the current admission period.

If the patient is discharged they are encouraged to create a new AHD at their earliest convenience.

The GoPC should be reviewed with the patient and updated for each new admission.

14. GoPC Form Validity period (MR00H.1)

The GoPC form is valid for the current admission but may be extended for a period of up to 12 months with appropriate consultant endorsement. In some sites, the consultant role will be fulfilled by the appropriate Senior Medical Officer.

For subsequent admissions and to avoid unnecessary duplication, the Senior Medical Officer will need to view previous GoPC summaries to check if the form is valid for 12 months and confirm with the patient/authorised person that the goals of care indicated is still current.

15. GoPC Amendment process (MR00H.1)

If the patient's clinical condition and treatment goals change then a new form needs to be completed. In these cases, the Senior Medical Officer must place a line through the old form,



date, sign and print name but leave it in the patient's medical record, behind the most current form.

Details of the reasons for the change to the GoPC Form must be documented in the medical record by the Senior Medical Officer.

16. Roles and responsibilities

16.1 Medical roles and responsibilities

Senior Medical Officers on the treating team are responsible for completing Section 1 of the GoPC.

- **Medical Officers** are responsible for:
 - alerting the nursing staff / shift coordinator / or nurse looking after the patient that GoPC have been identified and what has been decided (if they were not already involved in the process),
 - advising nursing staff / shift coordinator / or nurse looking after the patient of the patient's treatment preferences as outlined in the GoPC Form
 - review the GoPC as part of the clinical review process within the current admission or time period for which the form is valid.
- **A Senior Medical Officer** is responsible for:
 - facilitating the GoPC discussion.
 - listening and responding to the patient/family/carer's questions.
 - initiating timely discussions around treatment options, treatment-limiting orders and non-beneficial treatments to enable the patient/the person responsible to make an informed GoPC decision.
 - accurately reflecting the patient's wishes, values and preferences in the GoPC Form.
 - timely completion of the GoPC Form for relevant patients (Sections 2 and 3, and Section 4, if appropriate/relevant).
 - ensuring the form is signed.
 - Handwritten forms and printed eForms must be filed at the front of the patient's Medical Record to ensure prominent placement and easy access.

If the patient's condition changes, necessitating a new GoPC plan, then a new form is to be completed in a timely manner.

Forms (paper or eForm) cannot be amended on the physical copy. A new form must be completed, and the new version placed accordingly.



16.2 Nursing roles and responsibilities

Nurses provide care as identified within the GoPC plan.

Nurses participate in the GoPC meetings/discussions with patient, family, carer, Guardian.

Nurses have knowledge of the GoPC policy, GoPC procedure and GoPC Form and the stated requirements for patients in their care.

Nurses follow the patient's wishes, values and preferences as identified in the GoPC Form when required to respond to clinical deterioration / or medical emergency.

Nurses and members of the Multidisciplinary Team (MDT) provide feedback to the Medical Officer) regarding GoPC Form as required.

16.3 Multidisciplinary roles and responsibilities

Nursing and other members of the MDT, although not responsible for completing the GoPC Form, are responsible for contributing to the process and providing input into the decisions outlined in the form as necessary.

MDT may support the Medical Officers with patient and family discussions, provision of information and decision making.

16.4 Administrative roles and responsibilities

Clerical staff maintaining the patient's medical record should ensure the most current GoPC Form is filed at the front of the patient's medical record within the Alert divider (if present) and in accordance with Australian Standard AS2828.

17. Clinical Handover responsibilities

It is the responsibility of all clinical staff involved in the handover of patient care at any stage of the patient's journey to include information on the details of the GoPC in the handover including ambulance officers, clinical staff from other WA Health sites and external private organisations where the handover of clinical care occurs, in accordance with the requirements of the [WA DoH Clinical Handover Policy](#).

It is expected that all healthcare professionals respect and comply with the agreed GoPC.

Nursing staff will update GoPC instructions in iSoft and include this information at Clinical Handovers (ISOBAR) and reviews.

The PSOLIS Care Plan will identify the GoPC where treatment responses have been limited.

Medical Officers will include GoPC details in discharge summaries and forward a copy of the GoPC Form to the patient's GP and other discharge services as relevant.

18. Compliance

Failure to comply with this procedure may constitute a breach of the WA Health Code of Conduct (Code).

The Code is part of the [Integrity Policy Framework](#) issued pursuant to section 26 of the [Health Services Act 2016](#) (HSA) and is binding on all NMHS staff for which this purpose includes trainees, students, volunteers, researchers, contractors for service (including all visiting health professionals and agency staff) and persons delivering training or education within NMHS

NMHS staff are reminded that compliance with all policies is mandatory and it is the responsibility of line managers to ensure that all staff receive information about policies and policy updates.

19. Record Management

GoPC documentation is managed in accordance with the requirements of the [NMHS Information Management: Healthcare Record Management Policy](#).

20. Monitoring and evaluation

Monitoring and evaluation of the GoPC Procedure is achieved through adherence to the Audit Calendar requirements for MHPHDS MHS. Evaluation, audit and feedback processes are to be in place and compliance is monitored as part of clinical documentation audit calendar.

The Director of MHPHDS Safety & Quality and Performance Unit is responsible for overseeing the development of a standardised audit and monitoring tool and for supervising annual monitoring, frequency of monitoring, and reporting compliance.

Clinical incidents related to Goals of Patient Care are reported via Datix Clinical Incident Management System (Datix CIMS) and reports will be tabled at MHET Clinical governance Committee.

Related internal policies, procedures and guidelines
WA DoH Clinical Handover Policy NMHS Goals of Patient Care Policy NMHS Information Management: Healthcare Record Management Policy WA Health Consent to Treatment Policy – MH0175/22

References
Western Australian Legislation or Commonwealth legislation. Guardianship and Administration Act 1990

[Civil Liability Act 2002](#)

[Criminal Code \(Compilation Act 1913\)](#)

[Acts Amendment \(Consent to Medical Treatment\) Act 2008](#)

[National Safety and Quality Health Service Standards 2019](#)

Supportive & Palliative Care Indicators Tool (SPICT™). Edinburgh Scotland [Internet].

Available from: <https://www.spict.org.uk/>. [Accessed: 27 March 2020].

Government of Western Australia Department of Justice: [Office of the Public Advocate Making Treatment Decisions](#).

Useful resources

The GOPC Form (MRH001)

Policy Sponsor	MHPHDS Director of Nursing				
Policy Contact	MHPHDS Director of Nursing				
Date First Issued:	16/06/2023	Last Reviewed:	N/A	Review Date:	16/06/2026
Approved by:	MHPHDS Director of Nursing			Date:	31/03/2023
Endorsed by:	MHPHDS Executive Director			Date:	16/06/2023
NSQHS Standards Applicable:	☐  Std 1: Clinical Governance ☒  Std 2: Partnering with Consumers ☐  Std 3: Preventing and Controlling Healthcare Associated Infection ☐  Std 4: Medication Safety		☒  Std 5: Comprehensive Care ☒  Std 6: Communicating for Safety ☐  Std 7: Blood Management ☒  Std 8: Recognising and Responding to Acute Deterioration		
National Standards for Mental Health Services	☒ Std 1: Rights and Responsibilities ☐ Std 2: Safety ☐ Std 3: Consumer and Carer Participation ☐ Std 4: Diversity Responsibility ☐ Std 5: Promotion and Prevention ☐ Std 6: Consumers ☐ Std 7: Carers ☐ Std 8: Governance, leadership, and management		☐ Std 9: Integration ☐ Std 10: Delivery of Care ☐ 10.1 Supporting Recovery ☐ 10.2 Access ☐ 10.3 Entry ☐ 10.4 Assessment and Review ☐ 10.5 Treatment and Support ☐ 10.6 Exit and Re-entry		
Chief Psychiatrist's Standards for Clinical Care:	Aboriginal Practice, Assessment, Care Planning, Consumer and Carer Involvement in Individual Care, Physical Health Care of Mental Health Consumers, Risk Assessment and Management, Transfer of Care				
Printed or personally saved electronic copies of this document are considered uncontrolled					

Document Version Control			
Date	Version	Author	Notes
16/06/2023	1.0	Director of Nursing MHPHDS	Initial endorsement.

The health impact upon Aboriginal people have been considered, and where relevant incorporated and appropriately addressed in the development of this health initiative (ISD Number 606)

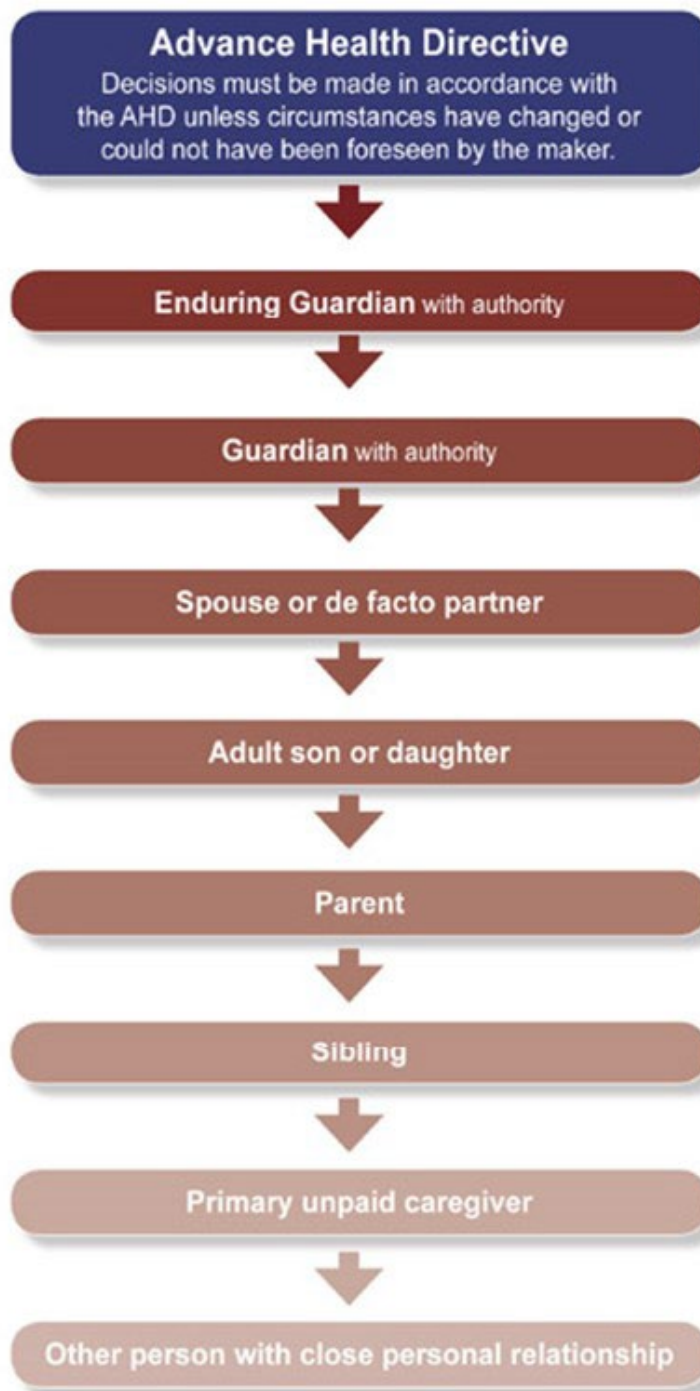
This document can be made available in alternative formats on request for a person with a disability.

© North Metropolitan Health Service 2021

Copyright to this material is vested in the State of Western Australia unless otherwise indicated. Apart from any fair dealing for the purposes of private study, research, criticism, or review, as permitted under the provisions of the *Copyright Act 1968*, no part may be reproduced or re-used for any purposes whatsoever without written permission of the State of Western Australia.

Appendix A: Hierarchy of Treatment Decision Makers

Where an AHD does not exist or does not cover the treatment decision required, the health professional must obtain a decision for non-urgent treatment from the first person in the hierarchy who is 18 years of age or older, has full legal capacity and is willing and available to make a decision.



Appendix B: SPICT™ Tool and Guidance for Use



Supportive and Palliative Care Indicators Tool (SPICT™)



The SPICT™ is used to help identify people whose health is deteriorating. Assess them for unmet supportive and palliative care needs. Plan care.

Look for any general indicators of poor or deteriorating health.

- Unplanned hospital admission(s).
- Performance status is poor or deteriorating, with limited reversibility. (eg. The person stays in bed or in a chair for more than half the day.)
- Depends on others for care due to increasing physical and/or mental health problems.
- The person's carer needs more help and support.
- Progressive weight loss; remains underweight; low muscle mass.
- Persistent symptoms despite optimal treatment of underlying condition(s).
- The person (or family) asks for palliative care; chooses to reduce, stop or not have treatment; or wishes to focus on quality of life.

Look for clinical indicators of one or multiple life-limiting conditions.

Cancer

Functional ability deteriorating due to progressive cancer.
Too frail for cancer treatment or treatment is for symptom control.

Dementia/ frailty

Unable to dress, walk or eat without help.
Eating and drinking less; difficulty with swallowing.
Urinary and faecal incontinence.
Not able to communicate by speaking; little social interaction.
Frequent falls; fractured femur.
Recurrent febrile episodes or infections; aspiration pneumonia.

Neurological disease

Progressive deterioration in physical and/or cognitive function despite optimal therapy.
Speech problems with increasing difficulty communicating and/or progressive difficulty with swallowing.
Recurrent aspiration pneumonia; breathless or respiratory failure.
Persistent paralysis after stroke with significant loss of function and ongoing disability.

Heart/ vascular disease

Heart failure or extensive, untreatable coronary artery disease; with breathlessness or chest pain at rest or on minimal effort.
Severe, inoperable peripheral vascular disease.

Respiratory disease

Severe, chronic lung disease; with breathlessness at rest or on minimal effort between exacerbations.
Persistent hypoxia needing long term oxygen therapy.
Has needed ventilation for respiratory failure or ventilation is contraindicated.

Other conditions

Deteriorating and at risk of dying with other conditions or complications that are not reversible; any treatment available will have a poor outcome.

Kidney disease

Stage 4 or 5 chronic kidney disease (eGFR < 30ml/min) with deteriorating health.
Kidney failure complicating other life limiting conditions or treatments.
Stopping or not starting dialysis.

Liver disease

Cirrhosis with one or more complications in the past year:

- diuretic resistant ascites
- hepatic encephalopathy
- hepatorenal syndrome
- bacterial peritonitis
- recurrent variceal bleeds

Liver transplant is not possible.

Review current care and care planning.

- Review current treatment and medication to ensure the person receives optimal care; minimise polypharmacy.
- Consider referral for specialist assessment if symptoms or problems are complex and difficult to manage.
- Agree a current and future care plan with the person and their family. Support family carers.
- Plan ahead early if loss of decision-making capacity is likely.
- Record, communicate and coordinate the care plan.

Please register on the SPICT website (www.spict.org.uk) for information and updates.

SPICT™, April 2019

Why use the SPICTM?

The SPICTM helps professionals identify people with general indicators of poor or deteriorating health and clinical signs of life-limiting conditions for assessment and care planning.

What will happen to each person and when is often uncertain. SPICTM looks at health status not a prognostic time frame. Identifying people with deteriorating health earlier improves care.

Using SPICTM to assess people's needs and plan care.

- After an unplanned hospital admission or a decline in health status: review current care, treatment, and medication; discuss future options; plan for managing further deterioration.
 - For people with poorly controlled symptoms: review and optimise treatment of underlying conditions, stop medicines not of benefit; use effective symptom control measures.
 - Identify people who are increasingly dependent on others due to deteriorating function, general frailty and/or mental health problems for additional care and support.
 - Identify people (and caregivers) with complex symptoms or other needs; consider assessment by a specialist palliative care service or another appropriate specialist or service.
 - Assess decision-making capacity. Record details of close family/ friends and any POA or Guardian for decision-making and involve them if the person's capacity is impaired.
 - Identify people who need proactive, coordinated care in the community from the primary care team and/or other community staff and services.
 - Agree, record, and share an Advance/ Anticipatory Care Plan; include plans for emergency care and treatment if the person's health (or care at home) deteriorates rapidly or unexpectedly.
- **Ask:**
- What do you know about your health problems and what might happen in the future?
 - *What matters* to you? What are you worried about? What could help with those things?
 - Who should be contacted and how urgently if your health deteriorates?
- **Talk about:**
- The outcomes of hospital admission and treatments such as: IV antibiotics; surgery; interventions for stroke, vascular or cardiac disease; tube or IV feeding; ventilation.
 - Treatments that will not work or have a poor outcome for this person. (e.g. CPR)
 - POA or Guardian for decision-making in case the person loses capacity in the future.
 - Help and support for family/ informal caregivers.